Sexual Assault

UP-22 | Universal Patient Care

Quick flow

01 Scene Safe + Private? [EMT / AEMT / Paramedic]

Separate from suspected assailant • request law enforcement for immediate danger • qualified interpreter

02 Treat Immediate Threats [EMT / AEMT / Paramedic]

XABCDE • hemorrhage • head injury • strangulation • intoxication/drugging • pregnancy • suicidal risk

03 Consent for Each Step [EMT / AEMT / Paramedic]

Explain before touching • obtain permission • patient may pause or decline any nonemergent step

04 Patient Younger Than 18? [EMT / AEMT / Paramedic]

Immediately report reasonable suspicion to DCS

05 Elderly / Vulnerable Adult? [EMT / AEMT / Paramedic]

Report suspected sexual offense to APS and local law enforcement

06 Competent Adult + Mandatory Exception? [EMT / AEMT / Paramedic]

Life-threatening injury • strangulation • knife, firearm, or other deadly weapon

07 Preserve Evidence Without Delaying Care [EMT / AEMT / Paramedic]

Minimal handling • separate paper bags • no forensic/internal exam or swabs • necessary emergency external assessment with consent

08 Appropriate SANE / Forensic Destination [EMT / AEMT / Paramedic]

Medical stability first • private handoff • document consent, reports, evidence, and patient preference

Overview

- Provide immediate medical care, safety, privacy, patient control, and trauma-informed support after a suspected or disclosed sexual assault.
- A competent adult may decline any assessment or treatment step and, unless a mandatory exception applies, may object to release of identifying information to law enforcement.
- Do not perform a forensic genital examination, internal pelvic or rectal examination, or forensic swabs under this protocol. When uncontrolled hemorrhage, traumatic injury, or severe pain makes external assessment necessary for emergency care, explain the need and obtain consent; expose only the area needed, preserve privacy and provide indicated trauma care. A patient with decision-making capacity may refuse all or part of the examination; honor that refusal. If capacity is absent and delaying necessary emergency care would risk serious harm, use only the applicable emergency-consent or minor-consent pathway. An emergency exception does not override a capable refusal.

Provider-Level Actions

Protect safety, consent, privacy, evidence, and access to appropriate medical and forensic care.

EMT

- Treat immediate threats and obtain only the history needed for medical care, safety, destination, and mandatory reporting.
- Explain each action before touching the patient, obtain consent throughout care, and allow the patient to pause or decline nonemergent assessment or treatment.

- Do not perform a forensic genital examination, internal pelvic or rectal examination, or forensic swabs under this protocol. When uncontrolled hemorrhage, traumatic injury, or severe pain makes external assessment necessary for emergency care, explain the need and obtain consent; expose only the area needed, preserve privacy and provide indicated trauma care. A patient with decision-making capacity may refuse all or part of the examination; honor that refusal. If capacity is absent and delaying necessary emergency care would risk serious harm, use only the applicable emergency-consent or minor-consent pathway. An emergency exception does not override a capable refusal.
- Preserve clothing or other potential evidence when feasible and transport to an appropriate medical and forensic-capable facility.

AEMT

- Perform all EMT actions. Provide authorized vascular access, fluid, analgesia, antiemetic, and monitoring care for associated illness or injury.
- Do not delay transport or compromise potential evidence for nonessential procedures.
- Continue consent checks and explain that medical treatment is available regardless of whether the patient chooses immediate law-enforcement involvement.

Paramedic

- Perform all prior care. Assess and treat serious injury, strangulation, airway or respiratory compromise, shock, head injury, overdose or drug-facilitated assault, pregnancy emergency, and suicidal risk.
- Select a medically appropriate SANE or forensic-capable destination when available and notify the receiving emergency department privately.
- Contact Medical Control for significant injury, questionable capacity, high-risk refusal, destination uncertainty, or conflict involving consent or mandatory reporting.

Tennessee Reporting Pathways

A competent adult's law-enforcement preference is respected unless a mandatory child, vulnerable-adult, or injury-reporting exception applies.

EMT

- Patient younger than 18: immediately report reasonable suspicion of sexual abuse to Tennessee DCS at 1-877-237-0004.
- Elderly or vulnerable adult: report the suspected sexual offense to both Tennessee APS at 1-888-277-8366 and local law enforcement.
- Competent adult: do not release identifying information to law enforcement over the patient's objection unless a statutory exception applies, including life-threatening injury, strangulation, or injury involving a knife, firearm, or other deadly weapon.
- For an injury covered by Tennessee Code §38-1-101, complete and document the EMS notification step: when transporting, privately tell the receiving physician or emergency-department staff the suspected cause of injury; when not transporting, report the result of the call to the 911 center through the approved agency channel. This does not replace separate required child-protection, adult-protection, or other legally required reports. Follow the approved safeguarding policy when reporting duties overlap.

AEMT

- Support immediate required reports while continuing patient care and preserving privacy from bystanders, family, and nonessential personnel.

Paramedic

- Resolve uncertainty through the agency safeguarding chain, Medical Control, and protective-service or law-enforcement consultation without delaying an immediately required report.

Indications

- Patient disclosure of sexual assault or concern that sexual contact occurred without consent.

- Concern for sexual assault based on injury, altered consciousness, drug-facilitated assault, caregiver or witness information, or scene findings.
- Request for emergency medical evaluation after a suspected sexual assault, regardless of whether the patient wishes to involve law enforcement.

Contraindications

- Do not perform a forensic genital examination, internal pelvic or rectal examination, or forensic swabs under this protocol. When uncontrolled hemorrhage, traumatic injury, or severe pain makes external assessment necessary for emergency care, explain the need and obtain consent; expose only the area needed, preserve privacy and provide indicated trauma care. A patient with decision-making capacity may refuse all or part of the examination; honor that refusal. If capacity is absent and delaying necessary emergency care would risk serious harm, use only the applicable emergency-consent or minor-consent pathway. An emergency exception does not override a capable refusal.
- Do not condition medical care or transport on the patient's willingness to speak with law enforcement or undergo a forensic examination.

Assessment | Immediate medical and safety assessment

- Assess scene safety, ongoing threat, need for law enforcement, airway, breathing, circulation, hemorrhage, head or facial injury, strangulation, chest or abdominal trauma, intoxication, overdose, and altered mental status.
- Assess for pregnancy-related emergency, suicidal thoughts, severe anxiety or dissociation, and the need for a support person or advocate chosen by the patient when safe and available.
- Consider drug-facilitated assault when there is unexpected sedation, memory loss, unexplained intoxication, or concern for covert drug administration. Provide time-sensitive receiving-facility notification.

Assessment | Consent and focused history

- Ask only what is necessary to guide emergency care, evidence-preservation advice, destination, and mandatory reporting.
- Explain each examination or procedure and obtain consent before proceeding. The patient may decline any nonemergent portion of care.
- Use the patient's exact words when documenting statements. Avoid judgmental language, conclusions, and repeated requests for a detailed account.

Assessment | Possible strangulation

- Assess voice change, dysphagia, dyspnea, neck pain or tenderness, petechiae, loss of consciousness, incontinence, seizure, confusion, focal neurologic symptoms, and memory gaps.
- Visible injury is not required for serious airway or vascular injury. Strongly recommend emergency-department evaluation and follow the mandatory injury-reporting pathway.

Treatment steps

- Ensure scene safety and treat immediate airway, breathing, circulation, hemorrhage, shock, head injury, strangulation, overdose, pregnancy emergency, or suicidal risk without delay.
- Create privacy when safe, use a qualified interpreter, explain every action before touching the patient, and obtain consent throughout care.

- Do not perform a forensic genital examination, internal pelvic or rectal examination, or forensic swabs under this protocol. When uncontrolled hemorrhage, traumatic injury, or severe pain makes external assessment necessary for emergency care, explain the need and obtain consent; expose only the area needed, preserve privacy and provide indicated trauma care. A patient with decision-making capacity may refuse all or part of the examination; honor that refusal. If capacity is absent and delaying necessary emergency care would risk serious harm, use only the applicable emergency-consent or minor-consent pathway. An emergency exception does not override a capable refusal.
- Preserve possible evidence without delaying care. When practical, advise the patient not to wash, change clothing, eat, drink, urinate, or brush teeth, but never withhold comfort or necessary treatment.
- If clothing is removed, minimize handling and place each item separately in a clean paper bag; do not use plastic for damp evidence. Document every item handled and transfer it according to agency evidence policy.
- Complete the required DCS report for a patient younger than 18 and both APS and local law-enforcement reports for a suspected sexual offense involving an elderly or vulnerable adult.
- For a competent adult, respect the patient's choice about law-enforcement involvement unless a mandatory reporting exception applies. Explain that medical and forensic evaluation may still be available without an immediate police report.
- Transport to a medically appropriate SANE or forensic-capable facility when available. Medical stability takes priority, and the final locally approved facility list remains part of the destination plan.
- For an injury covered by Tennessee Code §38-1-101, complete and document the EMS notification step: when transporting, privately tell the receiving physician or emergency-department staff the suspected cause of injury; when not transporting, report the result of the call to the 911 center through the approved agency channel. This does not replace separate required child-protection, adult-protection, or other legally required reports. Follow the approved safeguarding policy when reporting duties overlap. Document capacity, options, warnings, reports and the patient's decision.
- Treat pain, nausea, shock, and other medical problems under the applicable Claiborne protocol. Pregnancy prevention, STI prophylaxis, and HIV post-exposure prophylaxis are receiving-facility responsibilities.

Warnings / precautions

- Medical care and patient safety take priority over evidence preservation.
- Do not promise confidentiality when a mandatory report applies. Explain what information must be shared and with whom.
- Do not pressure a competent adult to involve law enforcement, undergo a forensic examination, or accept any nonemergent procedure.
- Do not allow the suspected assailant, coercive companion, or a minor child to serve as interpreter.
- A normal external examination does not exclude assault, internal injury, strangulation injury, pregnancy risk, or infection risk.

Clinical pearls

- Trauma-informed care returns control to the patient through privacy, explanation, choice, consent, and avoidance of unnecessary repetition.
- An adult may be eligible for a medical-forensic examination and Tennessee hold-kit pathway without making an immediate police report. Do not promise exact examiner availability before confirming with the receiving facility.
- Potential DNA evidence may be affected by washing or clothing changes, but prior washing, eating, drinking, urination, or time elapsed does not eliminate the value of medical or forensic evaluation.
- Drug-facilitated assault toxicology is time-sensitive; early receiving-facility notification is important.

Special populations | Children and adolescents

- Reasonable suspicion involving a patient younger than 18 requires immediate DCS reporting. Use developmentally appropriate, non-leading questions and avoid repeated interviewing.
- Use pediatric clinical dosing for patients younger than 16 years and the appropriate adult clinical pathway for patients 16 or 17 years old while maintaining mandatory child-reporting requirements through age 17.

Special populations | Elderly and vulnerable adults

- Report a suspected sexual offense to both APS and local law enforcement. Continue to involve the patient in decisions to the maximum extent supported by capacity.

Related pathways / contacts

Call Tennessee DCS - Child Abuse Hotline: 1-877-237-0004

Call Tennessee APS - Adult Protective Services: 1-888-277-8366

Source and references

Tennessee Code Annotated §§ 37-1-403 and 37-1-605 - child abuse and child sexual abuse reporting. | Tennessee Code Annotated § 71-6-103 - Adult Protection Act reporting. | Tennessee Code Annotated § 39-15-509 - reporting sexual offenses involving elderly or vulnerable adults. | Tennessee Code Annotated § 38-1-101 - reporting of certain injuries and first-responder duties. | Tennessee Office of Criminal Justice Programs - Best Practice Guidelines for Sexual Assault Response. | Tennessee Bureau of Investigation - Sexual Assault Kit and Hold Kit guidance. | Claiborne County EMS safeguarding, reporting, evidence-preservation, law-enforcement, destination, and refusal policies.

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